

Sample Medical Record - Case 2

DISCHARGE SUMMARY

Green Valley Medical Center – Pulmonary & Internal Medicine

PATIENT DEMOGRAPHICS

Patient ID: P982

Name: Maria L. Thompson

Date of Birth: 1958-11-03 (Age 67)

Sex: Female

MRN: 902144

Attending Physician: Dr. Kevin Morales, MD

Admission Date: 2026-06-02

Discharge Date: 2026-06-08

Length of Stay: 6 days

CHIEF COMPLAINT

Progressive shortness of breath, productive cough, and generalized weakness for one week.

HISTORY OF PRESENT ILLNESS

Ms. Thompson is a 67-year-old female with a history of chronic obstructive pulmonary disease (COPD), stage 3 chronic kidney disease, hypertension, and type 2 diabetes mellitus who presented to the emergency department with worsening dyspnea and productive yellow sputum. She also reported mild pleuritic chest discomfort and fatigue. Oxygen saturation on arrival was 87% on room air.

Initial evaluation raised concern for pneumonia versus pulmonary embolism. CT pulmonary angiography showed no evidence of pulmonary embolism. Chest radiograph demonstrated right lower lobe infiltrates consistent with community-acquired pneumonia.

PAST MEDICAL HISTORY

1. Chronic obstructive pulmonary disease (COPD)
2. Type 2 diabetes mellitus
3. Hypertension

4. Chronic kidney disease stage 3
5. Hyperlipidemia
6. Former smoker, 35 pack-year history, quit in 2021

ALLERGIES

Penicillin (hives)

Azithromycin (rash)

HOME MEDICATIONS ON ADMISSION

- Metformin 500 mg PO twice daily
- Losartan 50 mg PO daily
- Atorvastatin 20 mg PO nightly
- Tiotropium inhaler daily
- Albuterol inhaler as needed

HOSPITAL COURSE

The patient was admitted to the medical floor and started on intravenous ceftriaxone and azithromycin for treatment of community-acquired pneumonia. Azithromycin was discontinued on hospital day 1 after pharmacy review confirmed a documented allergy causing rash in the past. Doxycycline 100 mg twice daily was initiated instead.

The patient initially required 3 liters of oxygen via nasal cannula. Respiratory symptoms improved steadily over the hospitalization, and oxygen requirement decreased to room air by hospital day 5.

Creatinine on admission was elevated at 2.1 mg/dL above her baseline of approximately 1.5 mg/dL, concerning for acute kidney injury superimposed on chronic kidney disease. Intravenous fluids were administered cautiously with gradual improvement in renal function. Creatinine improved from 2.1 mg/dL to 1.6 mg/dL prior to discharge.

Blood glucose levels ranged from 160 to 240 mg/dL during hospitalization. Sliding-scale insulin was used temporarily while metformin was held because of acute kidney injury.

Pulmonology consultation recommended outpatient pulmonary rehabilitation and repeat pulmonary function testing.

PHYSICAL EXAM AT DISCHARGE

Temperature 36.7 C, blood pressure 128/74 mmHg, heart rate 78 bpm, respiratory rate 18, oxygen saturation 96% on room air.

General: alert, comfortable, no acute distress.

Lungs: mild bibasilar crackles, improved wheezing.

Cardiovascular: regular rhythm, no murmurs.

Extremities: no edema.

KEY LABORATORY RESULTS AT DISCHARGE

WBC: $8.4 \times 10^3/\mu\text{L}$

Hemoglobin: 11.2 g/dL

Platelets: $244 \times 10^3/\mu\text{L}$

Creatinine: 1.6 mg/dL

BUN: 28 mg/dL

HbA1c: 7.9%

Glucose: 172 mg/dL

DISCHARGE DIAGNOSES

1. Community-acquired pneumonia, improved
2. Acute hypoxic respiratory failure, resolved
3. Acute kidney injury on chronic kidney disease stage 3, improved
4. COPD exacerbation
5. Type 2 diabetes mellitus
6. Hypertension

DISCHARGE MEDICATIONS

- Doxycycline 100 mg PO twice daily for 5 days
- Losartan 50 mg PO daily
- Atorvastatin 20 mg PO nightly
- Tiotropium inhaler daily
- Albuterol inhaler as needed
- Insulin glargine 10 units nightly
- Hold metformin until PCP follow-up because of kidney injury

DISCHARGE INSTRUCTIONS

Complete antibiotic course as prescribed.

Monitor for worsening shortness of breath or fever.

Follow diabetic diet.

Avoid smoking exposure.

FOLLOW-UP APPOINTMENTS

Primary care physician within 1 week.

Pulmonology follow-up in 2 weeks.

Nephrology follow-up in 1 month.

CONDITION AT DISCHARGE

Stable condition with improved respiratory symptoms and improving renal function. Discharged home independently.